

--SUMMARY--

Decision No. 1517/19

15-Jun-2020

R.Nairn - P.Greenside - M.Ferrari

- Preexisting condition (back condition)

The worker had a significant pre-existing symptomatic back condition, which included many of the symptoms he complained of after the workplace accident. On a balance of probabilities, the contribution of the pre-existing back condition was sufficiently significant to render insignificant any contribution of the worker's compensable slip and fall to the worker's back condition, beyond the strain/sprain that was allowed under this claim.

The Panel found the worker did not have entitlement for the two claimed disc herniations, nor for a permanent impairment.

17 Pages

References: Act Citation

- WSIA

Other Case Reference

- [w3020s]k
- CROSS-REFERENCE: Decision No. 1517/19I

Style of Cause:

Neutral Citation: 2020 ONWSIAT 1152



WORKPLACE SAFETY AND INSURANCE APPEALS TRIBUNAL

DECISION NO. 1517/19

BEFORE:

R. Nairn : Vice-Chair
P. Greenside : Member Representative of Employers
M. Ferrari : Member Representative of Workers

HEARING:

April 29, 2020 at Toronto
Oral by Teleconference

DATE OF DECISION:

June 15, 2020

NEUTRAL CITATION:

2020 ONWSIAT 1152

DECISION(S) UNDER APPEAL: WSIB Appeals Resolution Officer (ARO) decision dated August 7, 2018

APPEARANCES:

For the worker:

Ms. C. Pelka, Paralegal

For the employer:

Mr. T. Kochanski, Lawyer

Interpreter:

Not applicable

Workplace Safety and Insurance
Appeals Tribunal

505 University Avenue 7th Floor
Toronto ON M5G 2P2

Tribunal d'appel de la sécurité professionnelle
et de l'assurance contre les accidents du travail

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REASONS**(i) Introduction**

- [1] At the time of the accident under consideration here, the worker was employed as a transport truck driver. The worker, born in 1993, was hired by the employer in about October 2016.
- [2] On November 19, 2016 the worker slipped and fell while walking back to his tractor trailer at a gas station. In his Report of Injury/Disease (Form 6) dated November 21, 2016 the worker indicated he injured his chest and upper and lower back. He described the mechanics of the accident as “fell onto left side of hip/butt. Landed on wet/muddy pavement. Extreme pain in lower back near spine, mid back and left side of chest”. In the Form 6 the worker also answered “yes” to the question “have you hurt this area(s) of your body before?”.
- [3] In its Report of Injury/Disease (Form 7) the employer described the areas of injury as “chest, left hip, right lower leg, both feet/toes and left buttock”. The Form 7 described the mechanics of the accident as follows:
- Driver states he was walking back to the tractor from the [gas] station. The pathway was cement turning into wet mud. He states he slipped backwards and fell on his Lt hip and buttock. He struggled to get up and walked back to the tractor; he also had difficulty to enter the tractor. The driver instructor drove to the [nearby] Hospital. States he was tested for reflexes. He is c/o chest pain when he takes a deep breath, tingling to the Rt side calf, soles of his feet and toes. States doctor suspects "hyperreflexia" but is unable to test him. He states he was given an injection for pain.
- [4] As indicated above, the worker was seen at a hospital emergency department on November 19, 2016 and the reporting of that assessment described the “presenting problem” as “back/spine – closed injury”. The report also noted “slipped from standing position while outside and landed in a ditch. C/O lower lumbar pain which radiates up rt side. States has some tingling in bilateral feet. Pt ambulatory since accident. States has hx of back issues with previous injuries”.
- [5] In a decision dated November 22, 2016 the WSIB (the “Board”) established a claim to deal with this accident and in a decision dated November 22, 2016 the worker was granted initial entitlement for “left foot, right toe(s) chest, left hip, right lower leg, right foot and left toe(s)” injuries. The worker was granted Loss of Earnings (“LOE”) benefits from November 20, 2016.
- [6] In a decision dated December 8, 2016, a Case Manager confirmed that the worker’s entitlement in the claim was being expanded to include health care benefits for a “low back sprain/strain type injury”. The Case Manager also noted that since there was “no medical imaging [confirming] a low back disc herniation” health care benefits were not granted for a “herniation diagnosis”.
- [7] Subsequently, arrangements were made to have the worker undergo an assessment at the Board’s Regional Evaluation Centre (“REC”) on January 26, 2017. In a report dated January 30, 2017 Dr. R. Shafer (the REC Medical Physician) indicated that given the worker’s clinical findings “including right lower extremity pain, numbness/tingling, asymmetric reflexes and motor weakness” an MRI of the thoracic spine was in order “to rule out neurological compromise and further direct plan of management”.

[8] As recommended by Dr. Shafer, the worker had an MRI of his thoracic spine performed on January 30, 2017. The results of this investigation were interpreted to reveal:

At T8-T9 no significant disc bulge is demonstrated. There is a broad-based central disc extrusion measuring 3 mm in A-P diameter, extending over 8mm in craniocardial dimension. This compresses the cord. There is no convincing evidence of myelomalacia.

At T9-T10 there is a slight left paracentral disc protrusion, measuring 1.5 mm in A-P diameter, extending over 4 mm in craniocaudal dimension. This approaches and potentially contacts the cord. There is no evidence of myelomalacia. The neuroforamina are patent bilaterally.

(...)

IMPRESSION:

Central disc herniation at T8-T9, compressing the cord, with a left paracentral disc herniation at T9-T10, again potentially contacting the cord. There is no evidence of myelomalacia (...).

[9] Dr. Shafer of the REC reviewed the results of the worker's MRI and in a report dated February 8, 2017 indicated in part:

[The worker's] thoracic spine MRI does not reveal a syrinx. It does however, reveal a central disc herniation at T8-T9, compressing the cord, as well as a left paracentral disc herniation at T9-T10, potentially contacting the cord. These findings likely correlate with and explain [the worker's] current clinical presentation, including back pain and right lower extremity neurogenic symptoms, with reported worsening of right lower extremity symptoms. [The worker] is a young and otherwise healthy individual with no history of significant back pain and no history of any lower extremity neurogenic symptoms. In the absence of any other known precipitating event, it is this Physician's opinion that [the worker's] lower thoracic MRI findings appear to be a direct result of the work-related injury in question, and likely correlate with his current clinical presentation. Given [the worker's] MRI findings of cord compression, with ongoing back pain and right lower extremity neurogenic symptoms, with worsening right lower extremity symptoms, it is this Physician's opinion that urgent referral to the WSIB Back and Neck Specialty Clinic for consultation with a Spine Surgeon would be recommended at this time in order to further direct plan of management, including treatment and return to work recommendations, as well as provide further commentary on prognosis and recovery timelines.

[10] After reviewing the comments of Dr. Shafer, the Board Case Manager asked for a medical opinion on the following issues:

1. Based on the accepted mechanism of injury (see below), is there a direct casual relationship between the central disc herniation at T8-T9, compressing the cord, as well as left paracentral disc herniation at T9-T10 and the 19Nov2017 slip and fall accident? Please explain rationale.
2. Noting the worker's extensive/ significant pre-existing, non-compensable conditions, is the worker's ongoing level of impairment directly related to the compensable condition, non-compensable condition, or both. Please explain rationale.
3. If so, what are the pre-existing condition(s) and is the contribution of the pre-existing condition(s) so great that it has rendered the compensable work related impairment insignificant?

[11] In a Memorandum ("Memo") dated April 3, 2017 Dr. G Seward (a Board Medical Consultant) replied:

The worker suffered a slip and fall injury on November 19, 2016 and sustained multiple soft-tissue injuries. The worker reports increased back pain and referral of symptoms into his right leg. The worker has attended for MRI of the thoracic spine and the report suggests a prominent central canal at T11-12, as well as at T5 through T6-7. The WSIB has suggested that the worker had no prior right leg symptoms and review of the prior clinical notes and records would confirm that the worker did not appear to have lower extremity symptoms. The worker is noted to have prior *severe* back pain related to his thoracolumbar scoliosis and has sought neurosurgical consult in 2015, however the scoliosis was deemed to be non-surgical.

Based on the accepted mechanism of injury, a slip and fall, there appears to be compatibility between the central disc herniation at T8-9 compressing the cord and the left paracentral disc herniation at T9-10. The reason for this opinion is that the worker has a significant scoliosis prior to this fall and the fall has likely placed enough strain on the spine to cause a disc to herniate. This is apparent as the worker does not appear to have prior radicular symptoms as noted in the clinical notes and records. Therefore, given the new findings of neurological difficulty in the lower limb, this would suggest a change in presentation following this incident and therefore the fall appears to be responsible for the central disc herniation and possible the left paracentral disc.

Based on the worker's pre-existing state, both the current work-related fall and the pre-existing painful scoliosis appear to be contributing to the current level of impairment. However the neurological compromise appears to have occurred post-incident and therefore has become the most pronounced area of concern. Therefore, though the pre-existing condition is likely contributing to the symptoms, the pre-existing condition would not appear to have rendered the accident-related impairment significant.

[12] After reviewing the evidence on file, the Case Manager issued a decision dated April 19, 2017 granting the worker entitlement for the “central disc herniation at T8-9 compressing the cord and left paracentral disc herniation at T9-10”. The Case Manager accepted that these diagnoses were compatible with the accident history.

[13] In May 2017 the worker was referred for further assessment at the Board’s Specialty Back & Neck Program. The worker was assessed on May 5, 2017 and in a report dated May 17, 2017 Dr. D. Wilson (orthopaedic surgeon) on behalf of the assessment team, indicated in part:

INTRODUCTION

[The worker] was assessed at the WSIB Specialty Back and Neck Program at Trillium Health Partners, Queensway site, on May 5, 2017. The purpose of the assessment was to perform a comprehensive musculoskeletal assessment of [the worker’s] work-related injuries to his Lumbar spine and to provide recommendations for further investigation and/or treatment to support the worker’s recovery, in addition to providing return-to-work recommendations.

(...)

Radiological Review Today

The Thoracic spine MRI was available on disc and was reviewed as noted above.

(...)

Clinical Impression

Diagnosis

This 24 year old male has a history of low back issues evident in the many investigations performed prior to this injury. His pain has intensified since the injury and treatment has not been effective to date.

Occupational Diagnosis: Thoracic/Lumbar spine strain/sprain.

Non-Occupational Diagnosis: T8-9, T9-10 disc bulges.

(...)

Return-to-work Restrictions

[The worker] is capable of returning to work. He should start on four hours per day and increase hours gradually to return to full duties over the course of eight weeks. He has reached maximal medical recovery. If permanent restrictions are required, these should be detailed as per a Functional Ability Evaluation.

Follow Up

None.

[14] The Case Manager reviewed the reporting from the Specialty Clinic and in a decision dated May 23, 2017, advised the worker that entitlement for the thoracic disc bulges was no longer in order. The Case Manager also advised that “furthermore, I am considering your compensable Thoracic/Lumbar spine strain/sprain injury to be fully resolved effective June 4, 2017 and there is no further entitlement under this claim beyond this date. As such, I will take no further action in your claim”

[15] The worker did not agree with the conclusions of the Case Manager. In the course of considering the worker’s objection, the Case Manager wrote to Dr. Wilson on March 6, 2018 and asked:

Further to your comprehensive assessment of [the worker] on May 5, 2017, I would like to confirm the following:

1. The identified T8-9, T9-10 disc bulges were specified as being a non-occupational while the occupational diagnosis was a Thoracic/Lumbar spine strain/sprain, this is correct? ·
2. If so please explain rationale as to why T8-9, T 9-10disc bulges were identified as being non-occupational.
3. If so, are the T8-9, T9-10 disc bulges a direct result of the worker's pre-existing, non-occupational back conditions?

[16] In a letter dated April 27, 2018 a Clerical Associate of the Specialty Clinic advised:

Please find the attached response (on the next page) from Dr. Wilson:

1. Yes
2. These are bulges to the annulus & not traumatic in origin
3. Yes

[17] In Memo No. A0057 of May 2, 2018, a Board Manager reviewed the information on file and concluded “I agree with the CM’s determination that the objective clinical evidence on file supports that the worker recovered from his work-related sprain/strain and any ongoing symptoms are a direct result of his non-occupational, pre-existing conditions”. Given the position taken by the Board Manager, the worker’s objection was eventually referred to an Appeals Resolution Officer (“ARO”) and in a decision dated August 7, 2018 the ARO denied the worker’s appeal. In so doing, the ARO concluded:

The WSIB Specialty Back and Neck Program report stated the worker reached MMR; no functional recovery was evident; and surgery was not an option. The worker was capable of returning to work and should start at four hours per day and increase his hours

gradually to return to full duties over the course of eight weeks. The report did not outline any restrictions, aside from the graduated return to work, nor is there direction for a Functional Abilities Evaluation (FAE), it is simply stated that if permanent restrictions are required, an FAE should be considered. However, based on the accepted diagnoses, I find a permanent impairment is not evident, even the REC had anticipated full recovery notwithstanding the results of the MRI, for which the findings have been determined to be non-compensable.

I note that in order to address the worker's representative's concerns pertaining to the non-occupational characterization of the central disc herniation at T8-9 and the left paracentral disc herniation at T9-T10, the Case Manager posed specific questions to the Dr. Wilson at the WSIB Specialty Back and Neck Program. The response from Dr. Wilson on April 27, 2018 confirmed that the T8-9 and T9-10 disc bulges were correctly specified as non-occupational; the rationale was that these are bulges to the annulus and not traumatic in origin; and it was confirmed that the disc bulges were a direct result of the worker's pre-existing, non-occupational back conditions. I find Dr. Wilson's rationale directly addresses the arguments posed by the worker's representative and I accept this expert opinion, which was based on a comprehensive review of all the medical information, including prior clinical records, and a physical examination of the worker.

Therefore, based on the totality of the evidence, the central disc herniation at T8-9 and the left paracentral disc herniation at T9-T10 are not allowed, and the medical information supports that the compensable thoracic/lumbar spine strain/sprain had resolved. In this case, it is far more likely that any ongoing problems are attributed to the worker's pre-existing, non-occupational back conditions.

CONCLUSION

The issues are concluded as follows:

1. There is no entitlement to the central disc herniation at T8-9 or the left paracentral disc herniation at T9-T10.
2. There is no ongoing entitlement/no permanent impairment

[18] The worker appealed the ARO's August 7, 2018 decision to the Tribunal.

(ii) Issues

[19] The issues to be determined in this appeal are:

1. Whether the worker ought to be granted entitlement in this claim for the disc herniations at T8-9 and T9-10 and,
2. Whether the worker ought to be granted ongoing entitlement (i.e. recognition of permanent impairment and a Non-Economic Loss ("NEL") determination) for his thoracic/lumbar spine conditions in this claim.

[20] The Workplace Safety and Insurance Appeals Tribunal supports the province-wide effort to stop the spread of COVID-19. In-person hearings were not possible during the time period for which this appeal was scheduled. A teleconference was offered as an alternative option to allow this appeal to proceed in a timely manner. In this case, it was confirmed that the parties consented to proceed by way of teleconference.

(iii) The worker's testimony

[21] Under questioning from his representative, Ms. Pelka, the worker confirmed that he started with the accident employer in about October 2016. He was 23 years of age and was working as a transport truck driver in training. He testified that his job duties included inspecting the tractor and trailer and driving both short and long haul routes.

[22] Prior to starting with the accident employer, the worker had spent 3 summers working for a local school board as a custodial assistant. In that job his duties involved helping to prepare and clean schools for reopening in the fall. The worker was also employed for about 8 months with a towing company. In that position, he was responsible for driving 20 foot shipping containers to various locations in Ontario and Quebec. The worker testified he had no problems performing either of these jobs.

[23] The worker testified that he had also completed a two year college automotive technician diploma program. He attended the school on a full-time basis. He obtained his DZ license in August/September 2015.

[24] The worker testified that prior to the accident in this claim, his overall health was good. He was questioned by his representative about prior issues with his back. The worker acknowledged he had experienced prior episodes of mid and low back pain. He noted that around the age of 14 years, he had been diagnosed with scoliosis or a "double S" curve in his spine. He described the pain from that condition as a "tightness" that he would experience in the area of his mid-back and left shoulder blade. He would have these symptoms once or twice a month and a flare-up could last 1 or 2 days. Physiotherapy was prescribed by his doctor and it was helpful in alleviating his pain. He might also take Tylenol or other over-the-counter medication when he had a flare-up. The worker found that the symptoms in his mid back would be increased by improper lifting. When that happened, he would take some Tylenol and then go on with his day. According to the worker, the pain he experienced in his mid-back after the 2016 accident was 3 to 4 inches lower than that he experienced before the accident.

[25] The worker recalled being referred to a neurosurgeon, Dr. Schacter, in 2015 because of his scoliosis. Dr. Schacter advised him that his scoliosis was mild.

[26] The worker testified that prior to his accident in 2016 he was an active individual. He was involved in soccer, swimming, skiing, fishing, snowmobiling and running.

[27] The worker described the mechanics of the accident on November 19, 2016. He testified that he slipped and fell onto his left side/hip area. He recalled experiencing "excruciating" pain from his buttocks up to his mid back and into the left side of his chest and ribs. He also experienced some tingling and numbness down his right leg. The worker was referred for physiotherapy which he underwent for about 6 months. When he stopped, his rib and back pain became "way worse". He testified this pain was different than that he had experienced before the accident. The centre of the pain was a few inches lower with radiation into his ribs and chest.

[28] The worker recalled having a return to work meeting with the employer at which time he was offered duties which included sitting in a vehicle in the yard filling out reports and helping with truck inspections. The worker declined the job offers noting his condition was such that he would not be capable of completing the commute to work which could take anywhere from 45 minutes to 2 hours. The worker never returned to work after the accident.

[29] The worker could not recall ever having been diagnosed with any disc herniations prior to his accident.

[30] The worker recalled being examined by Dr. Shafer of the REC in February 2017. Dr. Shafer recommended rest and physiotherapy and provided restrictions on lifting, bending, climbing and any unnecessary movements.

[31] The worker also recalled being referred to Dr. Wilson at the Board's Specialty Clinic. He described this assessment as "garbage". According to the worker, Dr. Wilson advised him he did not know why the worker had been sent there, given that he had already seen a surgeon. He testified Dr. Wilson examined him for 3 or 4 minutes and then told him he had a thoracic sprain and that he would "need to deal with it" since it could last the rest of his life. He was directed to go home and wait for his report. The worker testified that the entire visit to the Specialty Clinic lasted 35 to 45 minutes including the time spent waiting to see staff.

[32] The worker described the current condition of his back. He testified it is very painful and has resulted in his life being "totally different". He often has to lean on his elbows to reduce his back pain and it is difficult for him to sit for any length of time in a hard chair. He often lies on the floor. It is hard for him to bend and put his socks on. He has developed anxiety and depression. With the exception of some recreational swimming, his back pain prevents him from participating in any of his pre-accident hobbies. He sees his family physician (Dr. Johns) every few months. He cannot afford any therapy. He takes Tylenol for the pain. He was also prescribed Naproxyn but tries not to take it more than once a week. It helps him sleep. The worker continues to drive his car but only short distances. He is still technically employed with the accident employer but has never returned to work – his back and right leg pain keeps him from doing so. He does not believe he would be capable of returning as a truck driver because his pain would make it impossible for him to drive or even inspect his vehicle.

[33] The worker currently lives with and is supported by, his parents. He did not apply for Employment Insurance sick benefits or CPP Disability benefits because he thought he would make a quick recovery from his injuries. The worker advised he is having trouble coping with his diminished quality of life.

[34] Under questioning from Mr. Kochanski, the worker confirmed that in August 2015 he was about 6' 3" tall and weighed about 270 lbs. He advised he had prior problems with sleep apnea including snoring and night terrors. The worker acknowledged, as accurate, medical reporting which suggested he had been experiencing "chronic" low back pain prior to the accident in 2016. He acknowledged that Dr. Johns had referred him to an orthopaedic surgeon (Dr. Drew) and a neurosurgeon (Dr. Schacter), prior to the accident. He also confirmed that in August 2016 he had agreed to a referral to Dr. D. Dos Santos, a chiropractor, because the wait to see Dr. Drew exceeded two years and he wanted to be seen as quickly as possible.

[35] The worker acknowledged he had pre-existing scoliosis but testified it never stopped him from going to school and never affected his ability to work for more than a day at a time. He reiterated that the post-November 2016 pain he experienced was different than that he had experienced in prior years.

[36] The worker testified he has not looked for work since the accident.

[37] Under questioning from the Panel, the worker indicated that he initially enrolled in a college police training course but left after about a week. He then enrolled in the mechanics

course but never worked in that field. He obtained his DZ license and then found work with the towing company. He testified he did not have any difficulty performing those duties and did not lose any time from work. He left that job in about July 2016. He obtained his AZ license over the next 8 to 10 weeks and then in September 2016 had an interview with the accident employer.

[38] The worker testified that prior to the accident, he took only the occasional Tylenol or other over-the-counter medication to deal with his pain symptoms. Now, he takes regular Tylenol once or twice a week for pain. He was recently prescribed an anti-depressant. He cannot afford to fill his prescription for Naproxyn. The worker compared the nature of his pain pre and post November 2016. The mid back pain he currently experiences is 2 to 4 inches below where it was prior to the accident and rather than his pre-accident symptoms which felt like a “charley horse”, he now has a “horrible aching” in his mid back. His low back is no longer “manageable”.

(iv) Analysis

[39] Since this claim has an accident date in 2016, the applicable legislation is the *Workplace Safety and Insurance Act, 1997* (the “WSIA”).

(a) Entitlement for the T8-9 and T9-10 disc herniations

[40] In this case, the Board initially recognized that the worker sustained a variety of strain/sprain injuries in the accident on November 19, 2016 including a strain/sprain of his low back. An MRI of January 30, 2017 revealed disc herniations at T8-T9 and T9-T10 which the Board’s operating level determined should also be part of the worker’s entitlement in this claim following the opinion provided by their Medical Consultant, Dr. Seward. Entitlement to the disc herniations was subsequently overturned however, following the assessment by Dr. Wilson of the Board’s Specialty Clinic. The ARO confirmed the denial of entitlement for the disc herniations confirming that the condition was, more likely than not, related to the worker’s pre-existing non-compensable back condition.

[41] It is now well-settled in Tribunal case law that in dealing with matters of causation, the Tribunal employs a “significant contributing factor” test. In order for the worker to be successful in this appeal, it must be established on a balance of probabilities that the compensable accident of November 19, 2016 made a significant contribution to the T8-T9 and T9-T10 disc herniations. It is not necessary to establish that the compensable accident was the only contributing factor and entitlement would be in order even if there were a number of significant contributing factors as long as the compensable accident also made a significant contribution. It is worth noting that the Panel does not dispute that the worker is currently experiencing symptoms of back pain. The issue before us however, involves determining whether the compensable accident played a significant role in causing that back pain.

[42] In attempting to determine which factors played a significant role in the onset of the worker’s back pain, it is important, in our view, to have an accurate understanding of the worker’s pre-existing back condition. Under questioning from both his representative and the employer’s representative, the worker did not deny that he had experienced mid and lower back pain prior to the accident on November 19, 2016. The essence of the worker’s testimony, as we understood it, was that the pain he experienced before November 2016 and which was likely related to scoliosis that had been diagnosed when he was a teenager, was relatively mild. He compared the symptoms to a “tightness” or “charley horse” and indicated while he might experience these symptoms once or twice a month, they would resolve quickly – in a day or two

– with the use of over-the-counter Tylenol. The discomfort experienced, according to the worker, was never significant enough to keep him from working.

[43]

While we acknowledge the worker's testimony about the nature of his pre-existing condition, our review of the medical evidence satisfies us that the back problems he experienced prior to the accident in November 2016 were more significant than the worker described. In reaching that conclusion, we have taken particular note of the following:

- In a clinical note dated August 1, 2013 the worker's family physician noted the worker complaining of "chronic mid lower back pain & stiffness". Dr. Johns indicated the worker "lifted desk at work 3 weeks ago" and that "chronic pain has [been] progressively getting worse since then". Dr. Johns noted the worker had been "seeing the chiropractor for 2-3 years with minimal improvement". The worker's level of pain was such that it was "impairing quality of sleep and ability to work".
- In another clinical note of August 1, 2013 Dr. Johns noted that the worker started physiotherapy for "thoracic back pain". He was prescribed Lyrica.
- In his clinical note of August 13, 2013, it was noted the worker had a 4 week history of "progressively worsening pain" which, like the post-November 2016 pain, was "localized over L paraspinal and beneath scapula".
- In a clinical note dated September 29, 2014 Dr. Johns noted the worker with complaints of "chronic mid-low back pain over the last years – told he has double scoliosis in past". It was noted "pain makes standing for school difficult, OTC meds not helpful. Intermittent tingling in legs". Dr. Johns' assessment was "chronic [lower back pain] and scoliosis".
- In a clinical note of November 4, 2014 indicated that the worker was "anticipated to periodically miss some classes or leave them early because of a chronic back condition that limits his ability to sit or stand for long periods".
- In a letter dated November 19, 2014, Dr. Johns referred the worker to Dr. S. Lewis because of "worsening chronic mid-low back pain with past scoliosis diagnosis".
- On November 25, 2014 the worker underwent a "scoliosis study" because of what was described as "chronic low back pain".
- On February 11, 2015 the worker was seen by a neurosurgeon, Dr. I. Schacter (who had referred him for the scoliosis study). In the February 11, 2015 report which followed that assessment, Dr. Schacter noted in part:

The problem here is that he is having back pain which has increased and as you are well aware he has not had treatment for his scoliosis which was diagnosed at the age of about 13. He told me that he has constant pain in the back; it is very severe when he gets up in the morning and it takes about ½ hour to ease. He says that if he stands for a couple or hours or sits for an hour the pain in the lower back becomes much more pronounced. The back pain interrupts his sleep patterns. He also has what is noted to be a tremor in his upper limbs and I note that in the referral letter that he has been diagnosed as a familial tremor.

He is a big gentleman standing 6'1" in height and his weight is about 250 lbs. with very poorly toned musculature. I suggested to him that a weight reduction program and an active exercise program just for general conditioning would be helpful.

- The worker visited a hospital emergency department on March 24, 2015 with a presenting problem described as “back pain”. He was prescribed Toradol.
- In a letter dated March 25, 2015, Dr. Drew referred the worker to an orthopaedic surgeon, Dr. Drew noting:

I am hoping you can see [the worker] for a second opinion on his painful scoliosis condition. He has seen a neurosurgeon in Toronto for this (notes attached -there was no-one locally who would see him!). His pain is extreme, with his dysfunction having led to job loss and difficulty attending to school responsibilities. He is now taking narcotics to manage his pain, which when severe, causes total incapacity and vomiting.
- In his clinical note of December 17, 2015 Dr. Johns noted that the worker had a “sudden onset L chest wall pain 5 [days] ago in bed – worse to bend/twist/cough – pops at times [chest x-ray] negative in ER”. Dr. Johns concluded “likely rib/chest wall injury/strain [secondary to] scoliosis deformity”.
- As noted above, Dr. Johns had referred the worker to Dr. Drew for an assessment. In a letter dated August 26, 2016, Dr. Drew advised Dr. Johns given the significant wait times in his office and “in order to provide your patient with an option for a quicker spine assessment and consultation” a referral could be made to Dr. D. Dos Santos an “Advanced Practice Clinician with years of performing spine triaging of patients”. As the worker indicated in his testimony at this hearing, he agreed to take the referral to Dr. Dos Santos.

[44] Given the nature of the medical reporting referred to above, we find that the worker had a long-standing history of significant back pain with radiation into his legs. Dr. Johns, who had been the worker’s family doctor since he was 2 years old, described the worker’s back pain as “extreme” and having led to job losses and an inability to attend school. His pain was such that he had been prescribed narcotics to help address his “total incapacity”. The nature of the worker’s pre-existing back pain was such that he not only saw Dr. Johns repeatedly, but referrals to an orthopaedic surgeon and neurologists were also deemed necessary. In fact, in August 2016, just a few months prior to the compensable accident, the worker had agreed to a referral to Dr. Dos Santos.

[45] In addition to being satisfied that that the worker had a significant pre-existing symptomatic back condition, which included many of the symptoms he complained of after the events of November 19, 2016, our review of the post-accident medical reporting on file satisfies us, on a balance of probabilities, that the contribution of the pre-existing back condition was sufficiently significant to render insignificant, any contribution of the worker’s compensable slip and fall to the worker’s back condition beyond the strain/sprain that was allowed under this claim. In reaching that conclusion, we have taken particular note of the following:

- An x-ray taken on November 22, 2016 (with the worker only 23 years of age) revealed “mild degenerative changes throughout the thoracolumbar spine with slight convexity to the right of the lower thoracic and upper lumbar spine”.
- The worker saw Dr. Dos Santos on about January 6, 2017. In the report which followed that assessment, Dr. Dos Santos indicated in part:

History of presenting illness

[The worker] presents with a chief complaint of mid and lower back pain. The mid back pain is on the left side, while the lower back pain is midline. Pain is described as sharp,

with sharp and tight pain through the mid back, the upper back pain can refer to the left anterior chest wall. [The worker] reports his symptoms have been present since grade 10. There are no red flags such as changes to bowel or bladder function. There is some reported right leg weakness and numbness after a fall in November 2016. He did have physiotherapy a few years ago and is currently in physiotherapy again because of his fall at work.

(...)

Impression/Plan

[The worker] presents with a chief complaint of upper and lower back pain. Clinically there are signs and symptoms of mechanical back pain. This is likely related to facet/discogenic pain in the lower back. The upper back pain is related to costovertebral irritation with referral to the anterior chest wall. Based on the clinical presentation and examination findings, as well as a review of the MRI, there was no identified need to refer him on for spine surgical consultation. I have recommended to him that he engage in conservative therapy (...)

We note that Dr. Dos Santos, does not speak of any damage to the worker's spine nor does he relate the worker's symptoms which "have been present since Grade 10" to the compensable accident.

- One of the reports relied upon by the worker's representative was a June 9, 2017 report from Dr. Drew in which he indicated:

I am writing this letter to assist with [the worker's] claim.

Please note that [the worker's] mid and lower back pain started after a fall in the work place. This is what he reported to me at his consultation. Please be aware that I do feel his back pain was caused by the fall. I have attached my consult note with [the worker] for your review as well.

We find that we are able to give little weight to the opinion provided by Dr. Drew given that he does not appear to have had an accurate understanding of the worker's prior back condition. Dr. Drew notes that the worker "reported to [him] at his consultation" that his back pain started after his fall at work. As noted earlier in this decision, the Panel is satisfied that the worker had been experiencing significant back pain for a number of years prior to November 2016.

- Another report relied upon by the worker's representative at the hearing was a March 23, 2020 report from Dr. Drew in which he advised:

In my [June 9, 2017] letter to WSIB stated that I felt his mid and lower back pain started after a fall in the workplace. I did not specify the exact cause of his pain as this is unclear and impossible to be certain as to the exact pain generating structure. In my initial consultation letter, I stated it was possible his symptoms may be related to the disc herniations but alternatively it could be related to a thoracic strain. I also hypothesized his pain may be related to facet joint pathology as well. It is very difficult to link his mechanism of injury to disc pathology only. Although it is possible that his pain may be related some of his pain to the disc herniations in balancing all probabilities his mechanism is more consistent with a strain sprain that may involve his facet joints and or myofascial structures.

In our view, this report from Dr. Drew does not assist in establishing that the compensable accident of November 19, 2016 was a significant contributing factor in the worker's ongoing complaints of back pain.

Rather, Dr. Drew appears to contemplate a number of possible causes of the worker's pain, including those unrelated to the workplace accident. We are not persuaded that the possibility of some contribution supports a finding that it was a significant contributing factor.

- In his report of May 17, 2017 Dr. Wilson (an orthopaedic surgeon) of the Board's Specialty Back & Neck Clinic described the worker's T8-9 and T9-10 disc bulges as "non-occupational". The follow-up letter of March 6, 2018 provided further explanation indicating that they were "bulges to the annulus & not traumatic in nature". While we acknowledge the worker's comments and concerns about what occurred during his assessment at the Specialty Clinic, we note that Dr. Wilson's May 17, 2018 thorough report included a review of the accident history as well as the prior medical reporting and diagnostic testing on record. We give significant weight to Dr. Wilson's opinion noting that he is an orthopaedic surgeon and that he appears to have had an accurate understanding of the worker's prior back problems.
- In support of the appeal, the worker's representative referred to September 8, 2018 report from Dr. L. Prosia, a chiropractor. In that report, Dr. Prosia indicated in part:

[The worker] presented to my clinic on July 24, 2018 with a chief complaint of ongoing pain and disability as the result of a work-related accident on November 19th. While working as a truck driver he slipped and fell onto his left side and injured his back along with other areas of his body. [The worker] was assessed at the REC and had an MRI taken which revealed the presence of a broad-based central disc extrusion at T8-9 and a left paracentral disc protrusion at T9-10. The Case Manager sought a medical opinion and entitlements for these discopathies were accepted April 19, 2017.

(...)

The decision [to reverse entitlement] seemed to revolve around Dr. Wilson's rationale and explanation with regards to the disc herniations being "non-occupational" versus "occupational". It is stated in your letter to [the worker] that Dr. Wilson concluded, "the rationale was that these are bulges to the annulus and not traumatic in origin; and it was confirmed that these disc bulges were a direct result of the worker's pre-existing, non-occupational back conditions."

(...)

There are numerous flaws that are readily apparent with this line of reasoning. Firstly, [the worker's] injury at the T8-9 level is confirmed as an "extrusion" not a 'bulge' as incorrectly stated in Dr. Wilson's statement above in bold. This is a very important distinction in terminology because although extrusions can also happen over a period of time it is more likely that it is associated with a traumatic event whereby the nucleus pulposus actually breaks through the annulus fibrosis, whereby in a bulge it does not. Extrusions like protrusions typically require more force/trauma whereby bulges do not. The fall that [the worker] experienced onto his left side which was occupational in nature, is totally capable of delivering enough force to cause the extrusion seen on MRI. Bulges are a completely different matter and can definitely arise from non occupational situations.

Additionally, there is no possible way and it is ludicrous to think that Dr. Wilson can confirm that [the worker's] extrusion was "non-occupational" in nature. It is simply his unsubstantiated opinion that is not supported by any scientific or anatomical fact. The very fact that an objective imaging procedure (like a thoracic MRI) which is seen as the gold standard for evaluating the tissues in question, revealed significant damage in the area of [the worker's] spine where he fell onto, and the fact that his symptoms are

completely consistent with damage in this region affecting the thoracic nerve root and intercostal nerves, and the fact that [the worker] was able to work at his job as a truck driver without any difficulty prior to the fall makes it far more likely and credible that the injury seen in the 30/01 /2017 MRI is consistent with his work-related accident and is in fact "occupational". It should also be noted that there is a higher prevalence for disc herniations among people aged 30~50, with a male to female ratio of 2:1. My point here being that disc protrusions happen in younger people because of higher water content.

With all due respect to Dr. Wilson, these facts are considerably more in favour of an acceptance of [the worker's] claim than the unsubstantiated claims and opinion made by him.

Setting aside the tone of Dr. Prosia's report (in which he referred to Dr. Wilson's conclusions as "ludicrous", we find we are able to give it little weight and prefer the opinion provided by Dr. Wilson, an orthopaedic surgeon. It does not appear that Dr. Prosia had an accurate understanding of the severity of the worker's pre-existing back pain.

- In support of the worker's appeal, Ms. Pelka also referred to an April 7, 2019 report from Dr. M. Pysklywec of the Ontario Health Clinics for Ontario Workers Inc. In that report, Dr. Pysklywec indicated in part:

Impression:

[The worker] has had issues with his back for some time. The notes I have indicate back pain as far back as 2013 but it seems that he had it even before that when he a teenager. Things seemed to worsen with a work--related injury of 2016.

It is hard to reconcile his complete diagnosis at this point. He has thoracic back pain. This seems to be chronic as it was reported many years ago in the past. This was reported as severe. However [the worker] says that the pain went away and did not come back until the injury of November 2016. He also has a low back pain with numbness to the leg to suggest some radiculopathy. It is difficult to know what is causing that symptomatology. The lower lumbar imaging does not indicate a lumbar radiculopathy. EMG testing from February 2017 indicated some sort of neuropathy but insinuated that this does not relate to a spinal radiculopathy. In particular, Dr. Varey did not seem to think it related to the thoracic issue. From a clinical perspective, it is difficult to associate the T8-9 disc herniation causing radicular symptoms to the right leg.

(...)

With respect to the work-relatedness, it is challenging to sort this out as the diagnosis does not seem completely clear. It seems that there was chronic thoracic and even low back pain preceding the accident. It seemed to be quite severe at times as there was a repeated request for consultation as well as significant pain medications in the past. However [the worker] said that his symptoms went away and then came back with the injury.

The work-relatedness of the leg numbness and leg symptoms is challenging to comment on. I am not sure as to the origin of those symptoms. I would hope that further testing might help reveal this.

We interpret the reporting of Dr. Pysklywec to suggest, at best, that there is a possibility the worker's current back problems are related to the compensable accident in November 2016. It is well-settled in Tribunal case law that the possibility of a relationship is insufficient grounds upon which to grant entitlement. The evidence must establish that a relationship is more probable than not. It appears Dr. Pysklywec, like this Panel, also

questioned the worker's suggestion that he had been symptom free prior to the accident noting that his pain "seemed to be quite severe at times as there was repeated request for consultation as well as significant pain medication in the past".

- The decision of the Board's operating level to initially grant the worker entitlement for the disc herniations was based in part on the opinion provided by Dr. Shafer of the REC in his report of February 8, 2017 that the "lower back thoracic MRI findings appear to be a direct result of the work-related injury in question". While we acknowledge the opinion of Dr. Shafer, we find we can give it little weight. Dr. Shafer does not appear to have been aware of the worker's significant pre-existing back problems noting that the worker was "a young and otherwise healthy individual with no history of significant back pain and no history of any lower extremity neurogenic symptoms".
- Similarly, in granting the worker entitlement for the herniations, the operating level relied on Dr. Seward's memo of April 3, 2017. Dr. Seward concluded that both the pre-existing condition and the compensable accident were likely contributing to the worker's ongoing back problems. Like Dr. Shafer, Dr. Seward's conclusions are based in part on a finding that the worker had "no prior right leg symptoms". As noted earlier however, there is medical reporting from the worker's family physician that the worker had intermittent tingling into his legs from as early as September 2014.
- While the presence of the thoracic disc herniations was established by an MRI in the months after the accident, we do not have the benefit of any MRI testing conducted prior to November 19, 2016 which would conclusively establish whether the disc herniations were or were not present before the accident.

[46] In the Panel's view, the balance of evidence in this case establishes that prior to the accident on November 19, 2016 the worker was experiencing significant symptoms of mid and low back pain. The evidence establishes that just a few months before the accident, the worker's condition was such that he agreed to see Dr. Dos Santos rather than wait to see Dr. Drew. Before that appointment took place though, the worker slipped and fell at work. As Mr. Kochanski noted, there was nothing particularly significant about the mechanics of the fall in that the worker did not fall from a height nor did he strike anything on the way to the ground. He was initially diagnosed with sprain and strain injuries. X-rays of the thoracic spine revealed mild degenerative changes and an MRI of the lumbar spine on December 2, 2016 revealed "no significant disc pathology".

[47] Having had the opportunity to consider all of the evidence before us, the Panel finds that the T8-9 and T9-10 disc herniations that were eventually identified on an MRI were, more likely than not, part of the worker's pre-existing, non-compensable, back issues. Put another way, we find that the accident on November 19, 2016 did not make a significant contribution to the identified T8-9 and T9-10 disc herniations. We also find that the evidence for and against the worker's claim is not approximately equal in weight and therefore he is not entitled to the statutory benefit of doubt.

(b) Ongoing entitlement for the thoracic/low back strains

[48] As noted earlier in this decision, the Panel does not dispute that the worker has ongoing mid and low back pain. The most recent reporting from Dr. Prosia, the worker's chiropractor,

confirms that the worker is “incapacitated” by his pain and has not returned to work since the accident. However, as a review of Dr. Prosia’s September 8, 2018 report and the other medical reporting referred to earlier suggests, the worker’s ongoing complaints of back pain are, more likely than not, related to the T8-9 and T9-10 disc herniations which we have concluded are non-compensable rather than the mid and low back strains with which the worker was initially diagnosed close to four years ago. Given that the source of the worker’s back is non-compensable, he has no entitlement in this claim for a permanent impairment and NEL determination.

DISPOSITION

[49] The worker's appeal is denied.

DATED: June 15, 2020

SIGNED: R. Nairn, P. Greenside, M. Ferrari